

Poison Ivy - Oak - Sumac - Female (Adult)

Algorithm 1242 | Adult (18 years and older) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for suspected plant contact dermatitis, screening for anaphylaxis, eye/mucosal exposure, widespread blistering, infection and immune risk; not approved for production.
- Adult patient.
- Female.

Initial assessment

1. [LOCATION] Where is the rash?
2. [DURATION] How long ago was the plant contact?
3. [YES_NO] Any blistering or oozing?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant; are eyes, mouth, face or genitals involved; and are there breathing symptoms, fever, severe pain, widespread blistering, immune suppression, uncertain plant/chemical exposure or safeguarding concerns?

Triage and redirect questions

1. Redirect → Anaphylaxis

Are throat or tongue swelling, breathing difficulty, faintness, confusion, or collapse the main concern?

Systemic allergic-reaction features require the Anaphylaxis pathway.

1. Disposition level 100

Is there sudden swelling of the lips, mouth, throat, or tongue, or difficulty breathing along with the rash?

Universal emergency rule-out added ahead of the contact dermatitis guidance itself - severe allergic swelling or breathing difficulty requires immediate care, distinct from typical contact dermatitis.

1. Disposition level 50

Are symptoms persistent, recurrent, or severe, or has the triggering substance not responded to treatment?

NHS.UK guidance: arrange clinical review through a Qatar pathway approved by governance (GOVERNANCE_REQUIRED) for persistent, recurrent, or severe contact dermatitis symptoms.

1. Disposition level 50

Is this a mild, typical case with none of the features above?

NHS.UK guidance: a pharmacist can recommend emollients (moisturisers) for mild contact dermatitis.

Advice

Emergency allergic reaction precautions

- Call Qatar 999 and do not drive. Keep the patient lying unless breathing is difficult; use only their prescribed adrenaline auto-injector according to its plan and dispatcher instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: breathing worsens; swelling spreads or consciousness reduces.

Contact dermatitis follow-up

- Arrange in-person primary-care review through the Qatar pathway approved for UAT to confirm the cause and assess whether dermatology input is needed.
- Call Back If: symptoms worsen; not improving with treatment.

Review for suspected plant dermatitis

- Avoid further exposure, gently wash exposed skin and contaminated clothing, and arrange clinician/pharmacist review before creams or medicines are selected. Do not burn suspected plants because smoke exposure may be hazardous.
- Call Back If: rash worsens, spreads, blisters or becomes infected; eye, mouth, breathing or systemic symptoms.

First aid

- Call Qatar 999 and do not drive. Keep the patient lying unless breathing is difficult; use only their prescribed adrenaline auto-injector according to its plan and dispatcher instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Contact dermatitis", <https://www.nhs.uk/conditions/contact-dermatitis/> (page last reviewed 03 May 2023)

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 1b75027d164d98cd5b6f5f3b7c93ecc45e531b9eef912e9e5f4e0a15a94d82b9 | Status: BLOCKED_BY_RESEARCH_GAP